Medical Records Department Operations

Numbering, assembly, deficiency chase, filing, retrieval, retention and the destruction register.

Module	10 of 12
Study time	100 minutes
Audience	Medical records staff and officers, administrators, quality teams, clinical heads of department
Assessment	Knowledge check plus a 50-question scored bank
Standards basis	NABH 6th Edition IMS chapter; IMC Regulations 2002; state Clinical Establishments rules; DPDP Act 2023

The medical records department is the only part of a hospital whose entire output is the availability of information. Its performance reduces to two numbers: whether files are complete, and whether they can be produced when someone asks. Everything in this module — numbering, assembly, deficiency chase, filing, retention, destruction — exists to protect those two numbers.

LEARNING OUTCOMES — by the end of this module you will be able to:

- Describe the movement of a record from discharge through to permanent filing
- Compare numbering systems and explain why unit numbering is the accreditation expectation
- Explain terminal digit filing and why it reduces misfiles and congestion
- Operate a deficiency analysis and chase process with named ownership and escalation
- Apply a retention schedule that reconciles the professional minimum with longer legal and practical requirements
- Carry out a destruction process that is approved, screened, witnessed and permanently registered
- Measure MRD performance with indicators that drive action

10.1 The record’s journey

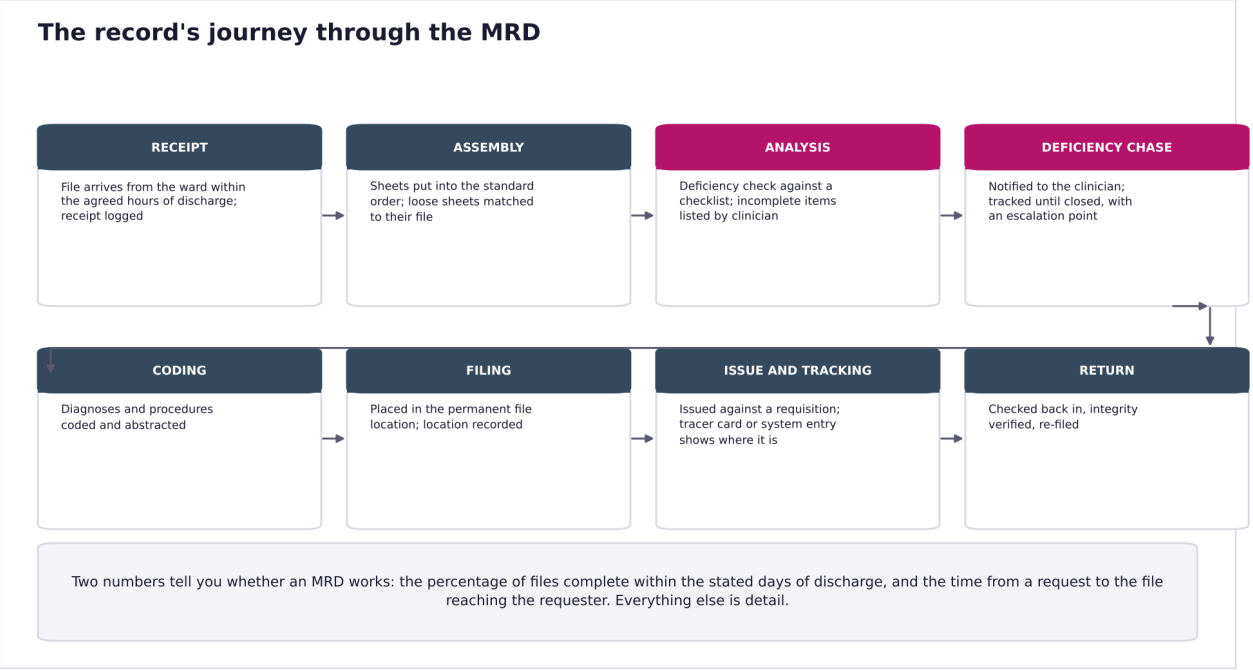


Figure 10.1 — Movement of a record through the medical records department.

Each step has a time standard, and each should be measurable. A hospital that cannot say how many files arrived in the department within twenty-four hours of discharge, or how many are still incomplete after fifteen days, is running the department on impression rather than on data.

Step	What it involves	A workable time standard
Receipt	File collected from or delivered by the ward, logged against the discharge list	Within twenty-four hours of discharge
Reconciliation against discharges	Every discharge has a file received; missing files chased the same day	Daily, with no carry-forward
Assembly	Sheets placed into standard order, loose sheets matched, pages checked for identifiers	Within forty-eight hours of receipt
Quantitative analysis	Deficiency checklist applied; missing documents and missing signatures listed by owner	Within seventy-two hours of receipt
Deficiency notification	The list sent to the named clinician and to the head of department	Within one working day of analysis
Deficiency closure	Clinician completes; the department verifies and closes	Within fifteen days of discharge
Coding and abstraction	Diagnoses and procedures coded and entered	Within the period the hospital's billing cycle requires, commonly seven days
Permanent filing	File placed in its permanent location and the location recorded	On closure of deficiencies
Retrieval on request	File located, issued against a requisition, tracked	Within the hospital's stated time, commonly a few hours for clinical requests

10.2 Numbering systems

Numbering and filing systems compared

System	How it works	Strength	Weakness
Serial numbering	A new number at every visit	Simple to issue; no lookup needed	One patient, many files; history scattered — unacceptable for continuity
Unit numbering	One number for life, all episodes in one file	Complete history in one place; the NABH expectation	Needs a reliable master patient index and duplicate-detection discipline
Serial-unit numbering	New number each visit, but old file pulled forward and merged	Works while migrating to unit numbering	Labour-intensive; gaps appear if the pull-forward is missed
Straight numeric filing	Files in plain numeric order	Intuitive for new staff	Activity concentrates at the high numbers; congestion and misfiling
Terminal digit filing	Filed by the last two digits, then the middle, then the first	Work spreads evenly across the shelves; misfiles easy to spot	Needs training; looks counter-intuitive until it is understood

The master patient index is the backbone of unit numbering. A duplicate MRN is a split medical history.

Figure 10.2 — Numbering and filing systems compared.

Unit numbering — one number allocated at first contact and used for every subsequent episode — is the accreditation expectation because it is the only system in which a clinician opening the file sees the whole history.

Serial numbering, in which a new number is allocated at each visit, produces a patient with six files in five places and makes continuity impossible.

The master patient index

Unit numbering only works if the index that maps a patient to their number is reliable. The master patient index must hold enough identifying information to distinguish patients with similar names, and it must be searched before a new number is issued. The commonest failure is a duplicate record created at registration because the search was skipped or the name was spelled differently.

- Search on more than name: date of birth or age, father's or spouse's name, address, phone number and any government identifier the hospital captures
- Standardise the capture of names, which in India vary in transliteration, order and the use of initials
- Run a duplicate-detection report regularly and merge duplicates under a controlled process with an audit trail
- Never delete a duplicate number; retire it, point it at the surviving record, and keep the mapping permanently
- Treat a duplicate as a patient-safety incident, because it is a split medical history

10.3 Filing

Terminal digit filing files by the last two digits of the record number, then by the middle pair, then by the first. Record 56-78-23 is filed in primary section 23, then under 78, then under 56. New numbers distribute evenly across all hundred primary sections instead of piling up at the end of the shelf, so work and staff spread evenly, shelf growth is predictable, and a misfile stands out because the last two digits of the neighbouring files differ.

- Colour-code the terminal digits on the jacket so misfiles are visible from a distance
- Use outguides or tracer cards for every file removed, showing who took it and when
- Audit a sample of shelves monthly for misfiles and measure the rate
- Keep the active and inactive files in separate areas, with a defined rule for when a file becomes inactive
- Protect against environmental risk: fire detection and suppression, water ingress, pest control, and controlled access — the record store is a high-value, irreplaceable asset

10.4 Issue, tracking and retrieval

A file that cannot be found is, for every practical purpose, a file that has been lost. Tracking is therefore not an administrative nicety: it is the control that makes the whole archive usable.

1. Every file leaves against a written or electronic requisition identifying the requester, the purpose and the date.
2. A tracer card or system entry replaces the file on the shelf, showing where it has gone.
3. A due-back date is set, and overdue files are chased on a schedule rather than when somebody notices.
4. Files move between departments only through the records department, never directly, so that the tracking record stays accurate.
5. On return, the file is checked for completeness and integrity before it is re-shelved.
6. The retrieval turnaround and the percentage of requests met are measured and reported.

> **CAUTION**
> Files held in wards, consultants' rooms and billing offices for weeks are the commonest cause of a file being unavailable when a patient returns or a legal request arrives. The department cannot fix this alone; it needs a hospital rule with an owner and an escalation.

10.5 Deficiency analysis and the chase

Quantitative analysis checks whether the required documents and signatures are present. Qualitative analysis, covered in Module 12, checks whether the content is adequate. Most hospitals do the first and call it record review.

The chase process is where most departments fail, because the deficiency list is sent to individuals who are busy and who face no consequence. The process works when the list goes to the head of department as well as the individual, when the hospital reports incomplete-record rates by unit at a management forum, and when there is a defined escalation — which in many hospitals includes withholding a privilege such as elective list allocation until records are completed.

> **KEY POINT**
> Measure deficiencies by unit, not only by individual. A unit with a persistent problem usually has a workflow cause — a rota that leaves nobody responsible for discharge summaries on a Saturday, or a theatre list that ends after the records clerk has gone home.

10.6 Retention

Retention — the minimum, and why hospitals keep longer

Record type	Common minimum in India	Practical hospital practice
Inpatient case records	3 years from the last entry (IMC Regulations 2002, Reg 1.3)	5 to 10 years; many corporate hospitals keep permanently in digital form
Medico-legal case records	No short limit; keep while any proceeding is possible	10 years minimum, often permanently; never destroyed while a case is live
Records of a minor	Limitation runs from majority	Until the patient turns 21, then apply the adult rule
Outpatient records	3 years where the state rules specify	3 to 5 years; longer where the OPD episode led to admission
Radiology images and films	3 to 5 years depending on modality and state rule	Digital archive retained with the record; films scanned before disposal
Death records and MCCD counterfoils	Permanent in practice	Permanent; indexed separately for quick retrieval
Operation theatre and anaesthesia registers	5 years commonly specified	Permanent, as they are the index to implant and procedure traceability

Where the state rule, the accreditation standard and the hospital policy disagree, the hospital must follow the longest period — and say so in writing.

Figure 10.3 — Retention: the professional minimum and what hospitals actually do.

The hospital's retention schedule must reconcile several sources: the IMC professional minimum of three years, the applicable state Clinical Establishments rules, limitation periods for civil claims, the special position of minors and medico-legal cases, accreditation expectations, and the DPDP Act's requirement not to keep personal data longer than the purpose requires. Where they conflict, the longer period applies and the reasoning should be written into the policy.

Category	Policy position	Reason
Medico-legal records	Retain for at least ten years, and permanently where a proceeding exists or is anticipated; never destroy while a case is live	Criminal proceedings have no comparable limitation, and these files are the ones demanded years later
Records of minors	Retain until the patient attains the age of twenty-one, then apply the adult rule	Limitation runs from the attainment of majority

Category	Policy position	Reason
Death records and certificate counterfoils	Permanent	Repeatedly required for succession, insurance and statutory purposes
Operation theatre and anaesthesia registers	Permanent	They are the index that makes implant and procedure traceability possible
Cancer and chronic disease records	Long-term or permanent	Continuity of care over decades and registry obligations
Records of patients in ongoing care	Retain while the episode of care continues, regardless of date	A chronic patient’s early records remain clinically relevant
Routine inpatient records	A defined period, commonly five to ten years, stated in policy	Balances limitation exposure and continuity against storage and data-minimisation duties
Outpatient records	A defined period, commonly three to five years	As above, with a longer period where the episode led to admission

Digitisation changes the storage problem but not the retention duty. A scanned record retained in place of the paper must be complete, legible, indexed, backed up, and produced in a form whose authenticity can be certified. The scanning process itself should be documented, including quality checks, so that the hospital can show that nothing was lost.

10.7 Destruction

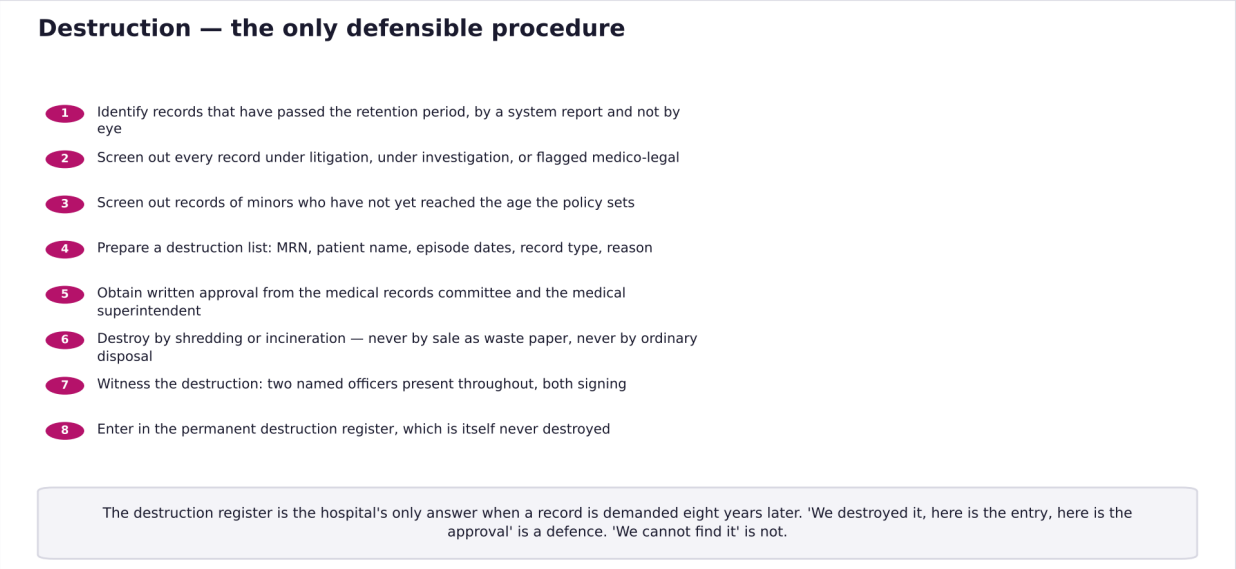


Figure 10.4 — The only defensible destruction procedure.

Destruction is the step where hospitals create their worst exposures, usually by doing it informally. Three screens must run before anything is destroyed: the retention period must have expired; the record must not be under litigation, investigation or a medico-legal flag; and the patient must not be a minor who has not yet reached the age the policy sets.

- Generate the candidate list from the system, not by looking at the shelf
- Screen against the medico-legal register, the litigation list and the research or registry hold list
- Prepare a destruction list recording medical record number, patient name, episode dates, record type and the reason for destruction
- Obtain written approval from the medical records committee and the medical superintendent before anything is destroyed

- Destroy by shredding or incineration; never by sale as waste paper and never into ordinary waste
- Witness the destruction: two named officers present throughout, both signing a certificate
- Record it in a permanent destruction register, which is itself never destroyed
- For electronic records, destruction means secure, irreversible erasure including backups and archives, with a certificate from the person who carried it out

> **KEY POINT**
> The destruction register is the hospital's only answer when a record is demanded years later. 'We destroyed it on this date, under this approval, with these witnesses' is a defence. 'We cannot find it' is not, and is often read as suppression.

10.8 Staffing, security and the physical department

The department must be sized to its workload and must not be the place where staff are posted when they cannot be used elsewhere. A medical records officer with professional training is a NABH expectation and a practical necessity, because the role involves legal judgment about release of information, not only filing.

- Access to the record store restricted to authorised staff, with a visitor log
- No record store that doubles as a thoroughfare or a general store room
- Fire detection and appropriate suppression, with records stored off the floor against water ingress
- Confidentiality undertakings signed by every member of staff with access, including housekeeping and contracted staff
- A business continuity plan for loss of the store or of the electronic system, including how the hospital would operate clinically
- Disaster recovery arranged for electronic records, with the restoration tested rather than assumed

10.9 Measuring the department

Indicator	How it is calculated	What it tells you
Files received within twenty-four hours of discharge	Files received in time / discharges	Whether wards release files promptly
Records complete within fifteen days	Records with no open deficiency at fifteen days / discharges	Whether the chase process works
Deficiency rate by item	Occurrences of each deficiency / records analysed	Which specific documents fail, and in which units
Retrieval turnaround	Requests met within the stated time / requests	Whether the archive is usable
Untraceable files	Files not located on request / requests	Whether tracking is real; the target is zero
Misfile rate	Misfiled records found / records sampled	Whether filing discipline is holding
Duplicate medical record numbers	Duplicates detected / new registrations	Whether the master patient index is being searched
Outstanding files held beyond due date	Files overdue / files issued	Whether the issue and return rule is enforced

10.10 Space, growth and planning

Record storage is one of the few hospital services with entirely predictable growth, and it is routinely planned only when the shelves are already full.

- Calculate linear shelf requirement from the average file thickness and the annual episode count
- Project growth over the retention period, not over the next year
- Plan the inactive-file area separately, with a defined rule for when a file becomes inactive
- Keep a working margin; a store that is physically full produces misfiles, because staff put files where they fit
- Plan the retrieval workflow and the counter area, not only the shelving
- Build in fire detection and suppression appropriate to paper storage, and keep records off the floor

10.11 Scanning and digitisation projects

Digitisation is attractive and frequently done badly. A scanned archive that cannot be searched, or in which pages are missing, is worse than the paper it replaced because the paper has usually been destroyed.

1. Decide the scope first: everything, or only active files, or only files from a given date forward.
2. Define the index. A scanned file retrievable only by record number, with no episode or document-type index, will be unusable for a specific request.
3. Set the quality standard — resolution, colour where clinically relevant, and legibility of handwritten entries — and audit a sample against the originals.
4. Count pages before and after, and reconcile. Missing pages are the commonest defect and the hardest to detect later.
5. Decide and document when the paper will be destroyed, and apply the full destruction process to it.
6. Establish backup, restoration testing, access control and audit trails before the paper goes, not afterwards.
7. Where the work is outsourced, bind the contractor to confidentiality, define where the data may be processed, and keep the right to audit.

> **CAUTION**
> Do not destroy the paper until the scanned record has been verified as complete and legible, retrievable, backed up and restorable. Destruction is irreversible and a scanning defect discovered afterwards cannot be remedied.

10.12 Records in a disaster

A fire, a flood or a ransomware attack can destroy a hospital’s entire archive. Business continuity for records is rarely planned and is always needed in exactly the circumstances when planning is impossible.

- A documented continuity plan stating how the hospital would function clinically without the archive
- Off-site or cloud backup for electronic records, with restoration tested rather than assumed
- Physical protection for the paper store: fire detection and suppression, water ingress protection, and storage above likely flood level
- A priority recovery list — which records would be recovered first, such as active inpatients and medico-legal files
- A paper-based clinical workflow that can run during an extended electronic outage, with the forms stocked
- A documented process for reconstructing records lost or damaged, with the reconstruction clearly identified as such
- A record of the disaster and of what was lost, which will be needed for every later request for a destroyed file

10.13 Multi-site hospitals and outsourced services

Where a hospital operates across sites, the patient must have one record number and one record, or the continuity benefit of unit numbering is lost. Where any part of the records function is outsourced — coding, scanning, storage, transcription — the hospital remains the custodian and remains accountable.

Arrangement	What the hospital must control
Shared master patient index across sites	A single index, with registration staff at every site trained to search it before issuing a number
Records physically held at one site	A retrieval and transport process with turnaround standards, and tracking that survives the journey
Outsourced coding	Access limited to what the coder needs, audit trails, confidentiality undertakings, and an accuracy audit of the vendor's output
Outsourced scanning or storage	Contractual confidentiality and security terms, defined processing locations, the right to audit, and a clear position on what happens at contract end
Outsourced transcription	The same controls, plus a review-and-authenticate step by the clinician before the transcript becomes part of the record
Cloud-hosted electronic records	Data location, access control, audit trail availability, backup and exit arrangements, assessed against the hospital's data protection obligations

10.14 The records policy manual

Everything in this module should exist as written, approved, current policy. An assessor will ask for the manual, and the content of the manual is the first test; whether practice matches it is the second.

- Scope: which records, which settings, which formats
- Contents and assembly order, with the forms list referenced
- Identification, authentication and correction standards
- Approved and prohibited abbreviation lists, maintained as controlled documents
- Deficiency standards, the chase process, and the escalation
- Numbering, filing, issue, tracking and retrieval standards with time targets
- Retention schedule by record type, with the reasoning
- Destruction procedure, approvals and register
- Release of information: who may approve, what verification is required, and the register
- Confidentiality, access control and breach reporting
- Record review and audit programme, with indicators and the reporting forum
- Business continuity and disaster recovery for records

10.15 Putting this module to work

1. Ask what proportion of files reached the department within twenty-four hours of discharge last month.
2. Pick a discharged patient from six months ago and request their file. Time how long it takes to arrive.
3. Look at the deficiency list for your unit. Does it name individual owners and is it escalated to the head of department?
4. Ask to see the destruction register. If there is none and records have been destroyed, that is the finding.
5. Check whether medico-legal files are flagged in a way that would exclude them from a destruction run generated by the system.

Knowledge Check

Work through each item before reading the model answer. These are applied questions: the answer is the standard of documentation your assessor, your medico-legal adviser or the next clinician to read the file would expect.

Q1. **A patient attending for the fourth time is given a new registration number because the clerk could not find the earlier record. What has gone wrong and what is the consequence?**

Answer: The master patient index was not searched adequately, or the index does not hold enough identifiers to find the patient. The consequence is a split medical history: allergies, previous investigations, prior admissions and implant records now sit under a different number and will not be seen. This is a patient-safety incident, not a clerical error. The duplicate should be detected, merged under a controlled process with an audit trail, and the retired number permanently mapped to the surviving record.

Q2. **Your hospital files records in straight numeric order and finds that one end of the store is congested while the rest is idle. What change addresses this, and how?**

Answer: Terminal digit filing. Files are placed by the last two digits of the record number, then the middle pair, then the first, so consecutive new numbers scatter across all hundred primary sections instead of accumulating at the high end. Work and staffing spread evenly, shelf growth is predictable, and misfiles become visible because the terminal digits of neighbouring files differ. It requires training, because it is counter-intuitive until it is understood.

Q3. **A hospital destroys records older than three years using a list prepared by a clerk from the shelves. A summons later arrives for a destroyed medico-legal file. Identify every failure.**

Answer: The candidate list was prepared by eye rather than from a system report, so completeness cannot be shown. There was no screening against the medico-legal register, the litigation list or minors' records. There is no evidence of committee and superintendent approval. There is no witnessed destruction certificate and, on these facts, probably no permanent destruction register. And the retention period applied was the professional minimum rather than the hospital's own reconciled schedule, which should never have permitted destruction of a medico-legal file.

Q4. **A consultant has held eleven files in his room for three weeks for a research project. What should the department do?**

Answer: The files are issued and overdue, so the tracking system should already have flagged them. The department chases through the defined escalation: a reminder to the consultant, then to the head of department, then to the management forum where overdue files are reported. Separately, research use of records requires ethics committee approval and a defined data handling arrangement, so the department should check that approval exists. In the meantime those files are unavailable if the patients return or a legal request arrives, which is the risk being carried.

Q5. **The MRD reports a retrieval turnaround of ninety-eight per cent but clinicians say files are never available. What might explain the gap?**

Answer: The indicator is probably measuring only requests the department could fulfil, excluding requests for files that are already out, untraceable or never received from the ward. It may also be measuring time to locate rather than time to deliver. A meaningful indicator counts every request, counts failures, and measures the time until the file reaches the requester. The untraceable-file count should be reported alongside it, since that is the number clinicians are actually experiencing.

Q6. **Your hospital scans all records and destroys the paper after one year. What must be in place for this to be defensible?**

Answer: A documented scanning process with quality checks confirming that every page was captured and is legible, indexing that allows retrieval of the complete episode, a retention schedule applied to the electronic record, secure backup and tested disaster recovery, access controls and audit trails, and the ability to certify the electronic output for evidential purposes. The destruction of the paper must itself follow the approved, screened, witnessed and registered process. Without the certification capability, the hospital has weakened its evidence position by destroying the paper.

ASSESSMENT BANK
A fuller bank of 50 scored questions for this module is supplied separately as a JSON question bank for loading into your LMS.